Global Health Spending



Rena · Coco · Ibrahim · Matthew · Yihan

01

Who pays for health care?

Government, private, and external aid.

02

When the citizen pays?

Out-of-pocket dependency and the state-versus-citizen burden.

03

Where does the money actually go?

Curative care, medical goods, prevention.

Data Set Overview

Financing Schemes

Who Pays?

- Government & Compulsory
- Voluntary Health Insurance
- Out of pocket payments
- External financing

Spending Purpose

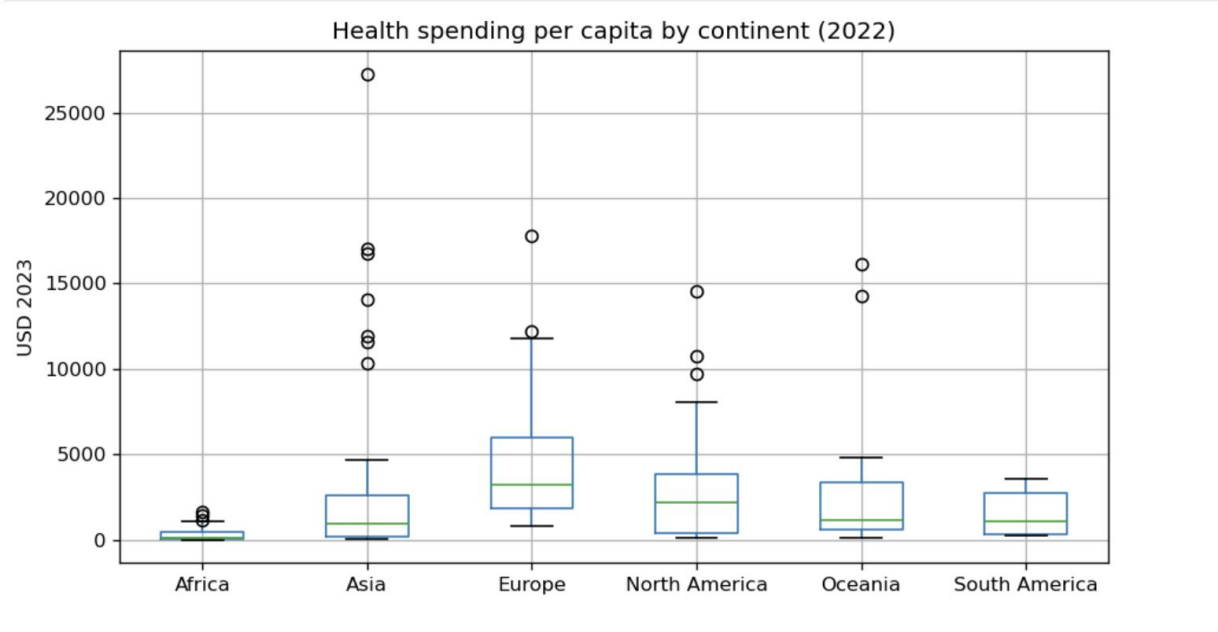
Where it goes?

- Curative/Rehabilitative Care
- Long-term care
- Medical goods
- Preventive Care & Governance.

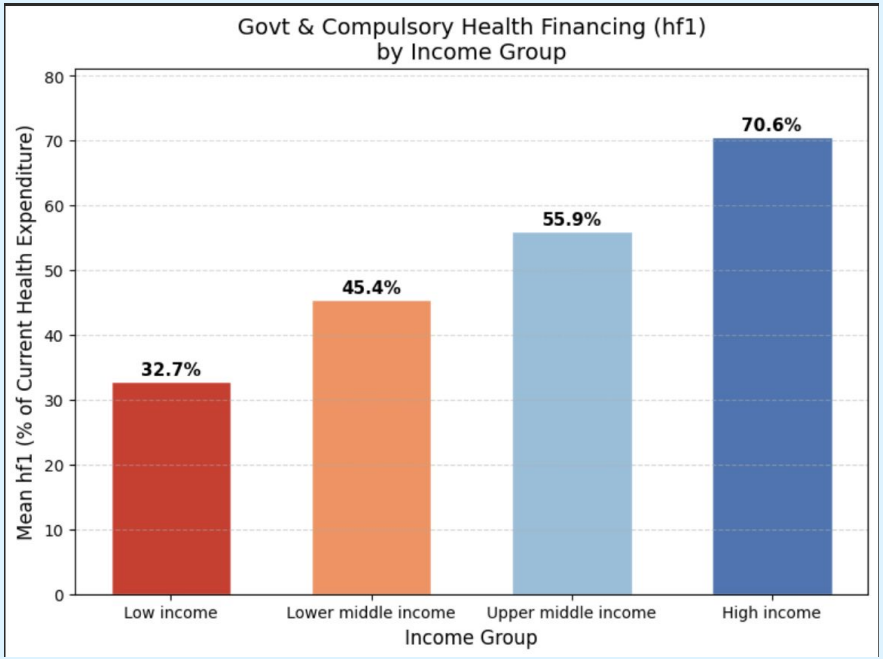
External Sources (Merged)

- Life Expectancy
- GDP per capita
- Population
- Income Group classification

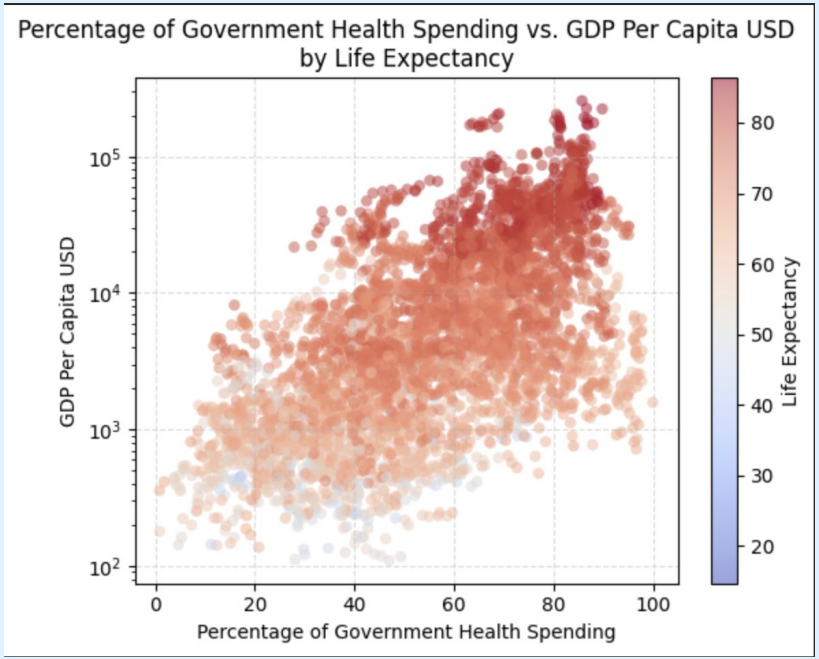
Overview per capita spending by continents



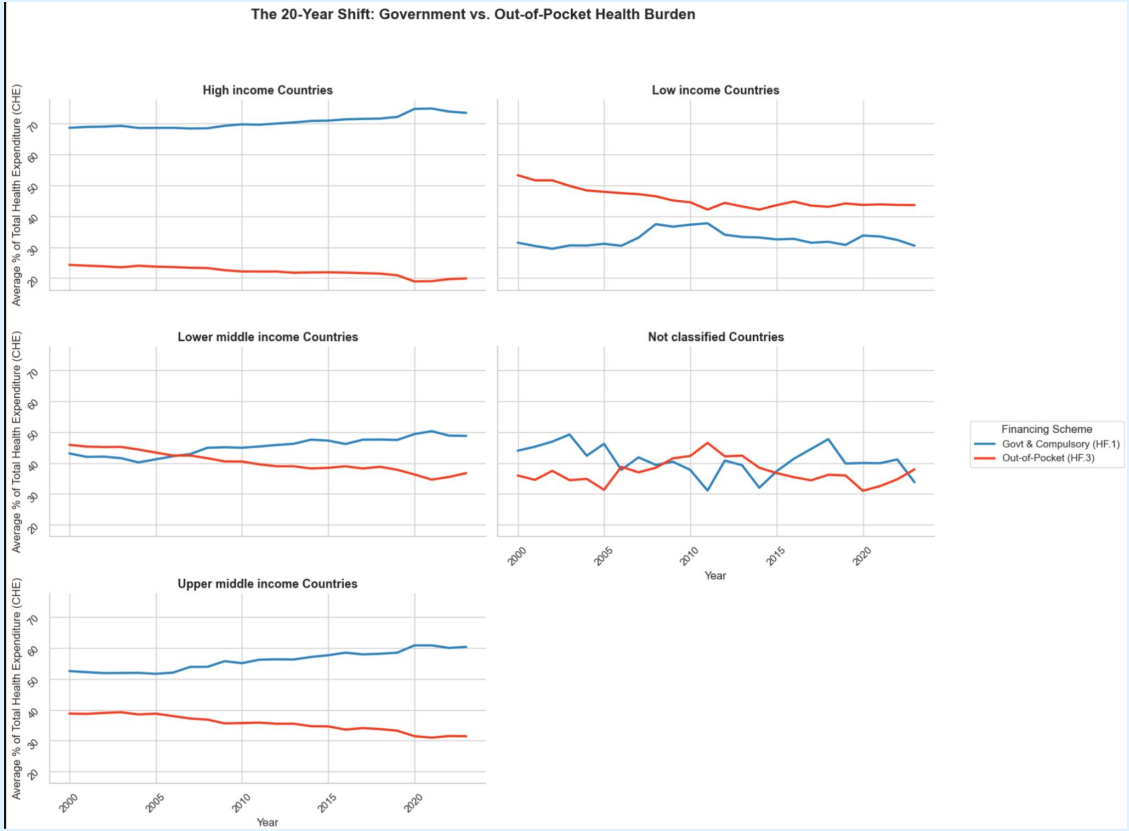
Financing Scheme



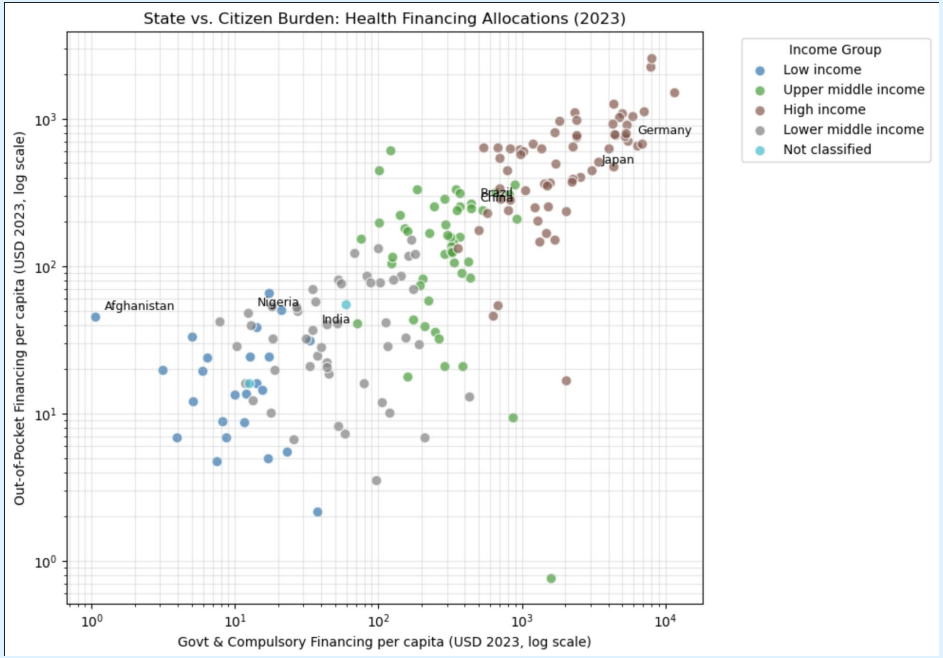
- As a country's income increases the percent of health spending from the government increases.
- Jump from low income to lower middle income larger than lower middle income and upper middle income.
- Even at high income, ~30% of funding comes from elsewhere.



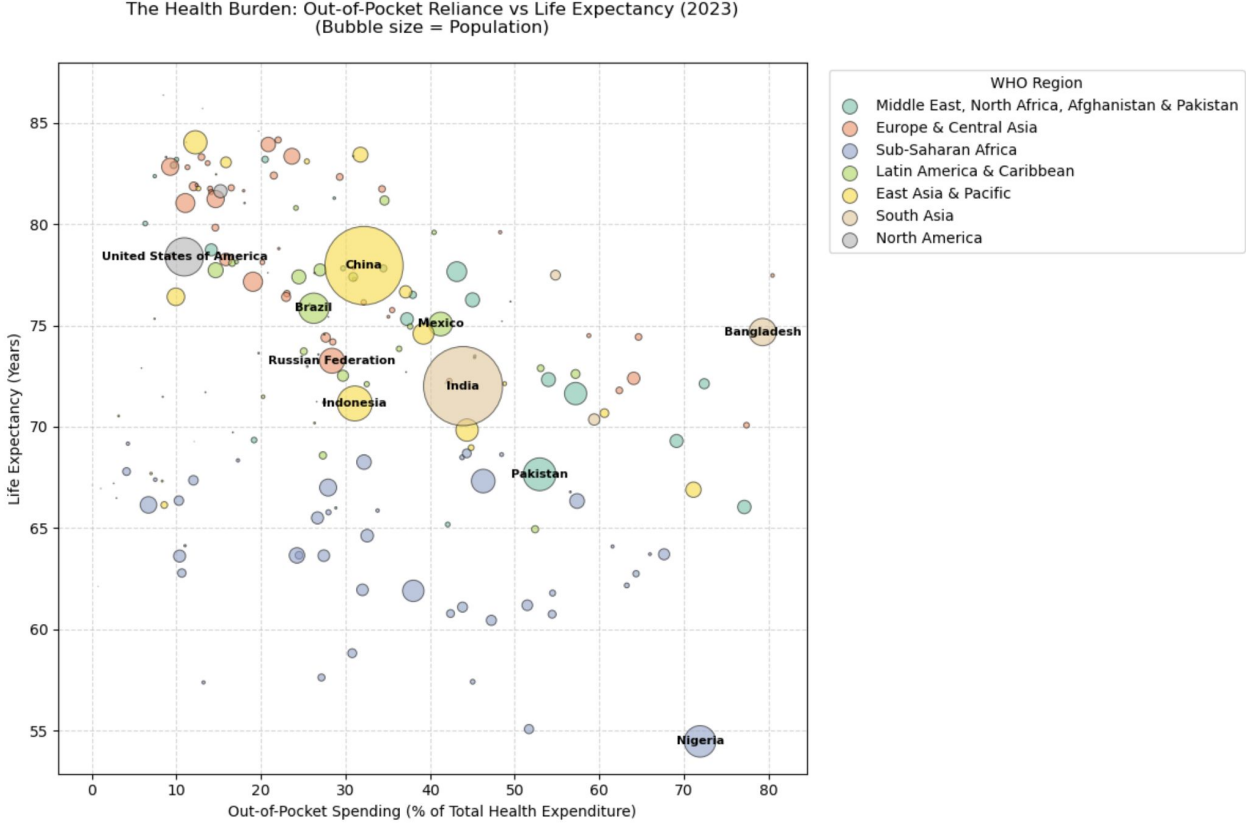
- Positive correlation between Percentage of Government Health Spending and GDP Per Capita (USD) and Life Expectancy.
- Lower percentage of Government Health Spending has more variation in GDP Per Capita (USD) and life expectancy. Another variable?



- The wealthier the nation, the larger the gap between out-of-pocket spending and government spending.
- Low income countries have to pay more out-of-pocket spending than the government spending.



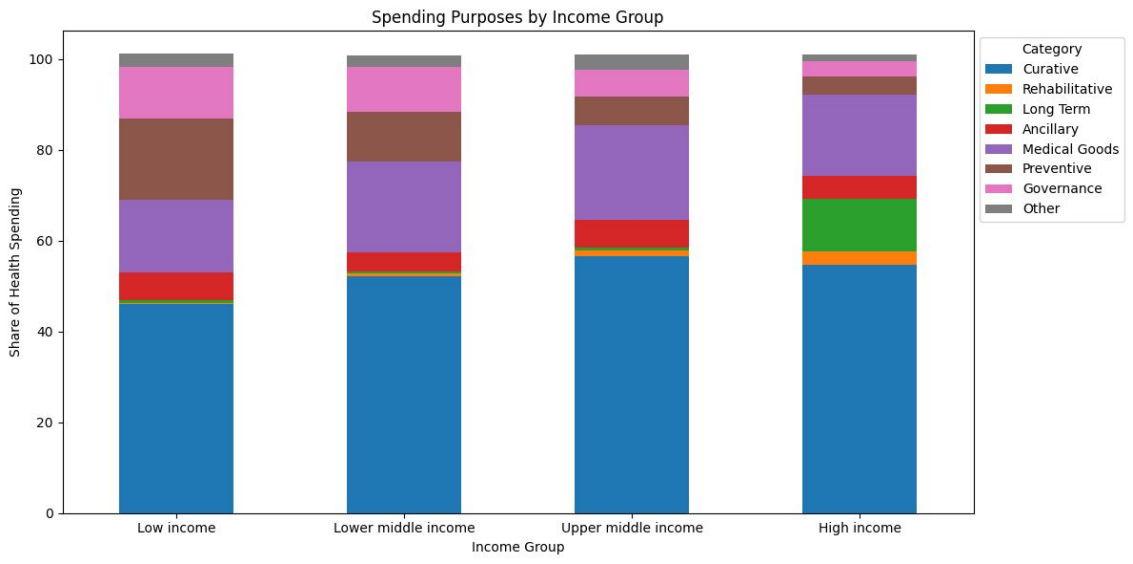
- Wealthier countries both have more government spending budget and out-of-pocket spending than poorer countries.



- High reliance on out-of-pocket financing is strongly correlated with lower life expectancy.
- Most European and Central Asian countries (including the US) both have less burden on out-of-pocket health spending and a higher life expectancy.
- Most African countries have comparatively lower life expectancies

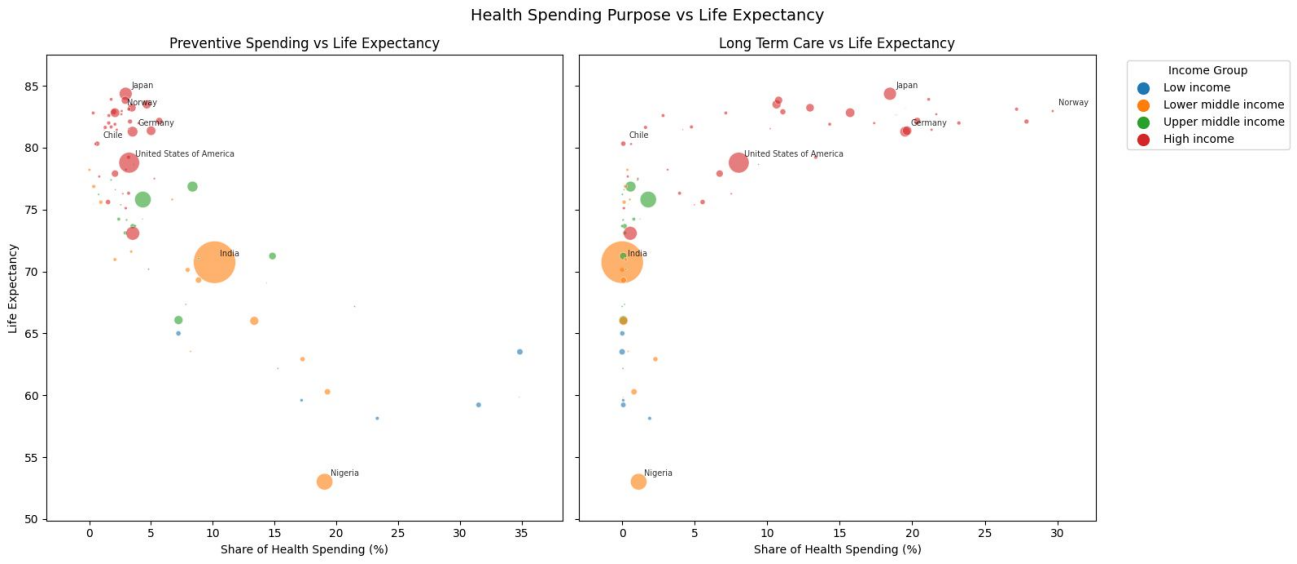
Spending Purpose

Higher income enables long-term spending



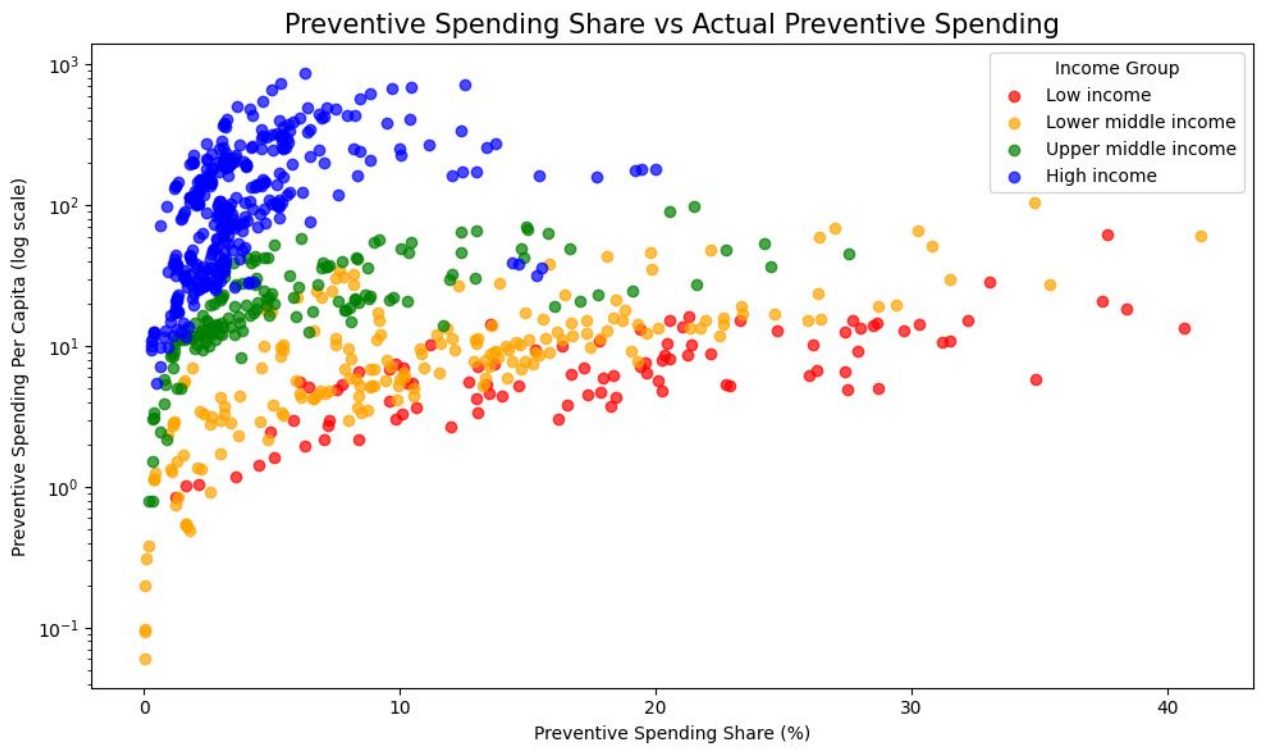
- Although spending in most categories is fairly consistent across income groups, **preventive care** and **governance** spending **decrease**, while **long term care** and **rehabilitation** costs **increase**
- This suggests that **as countries become wealthier**, greater spending ability for **long-term conditions** is enabled
- Conversely, prevention becomes less of a focus

Extending life expectancy motivates spending



- Countries spending **more on prevention** see **lower life expectancies**
- Lower-income countries make up a much larger share of this group
- Higher percentage of **long term care spending** is associated with **higher life expectancy**
- Spending in high income countries is focused on **extending life expectancy** countries rather than investment into **disease prevention efforts**

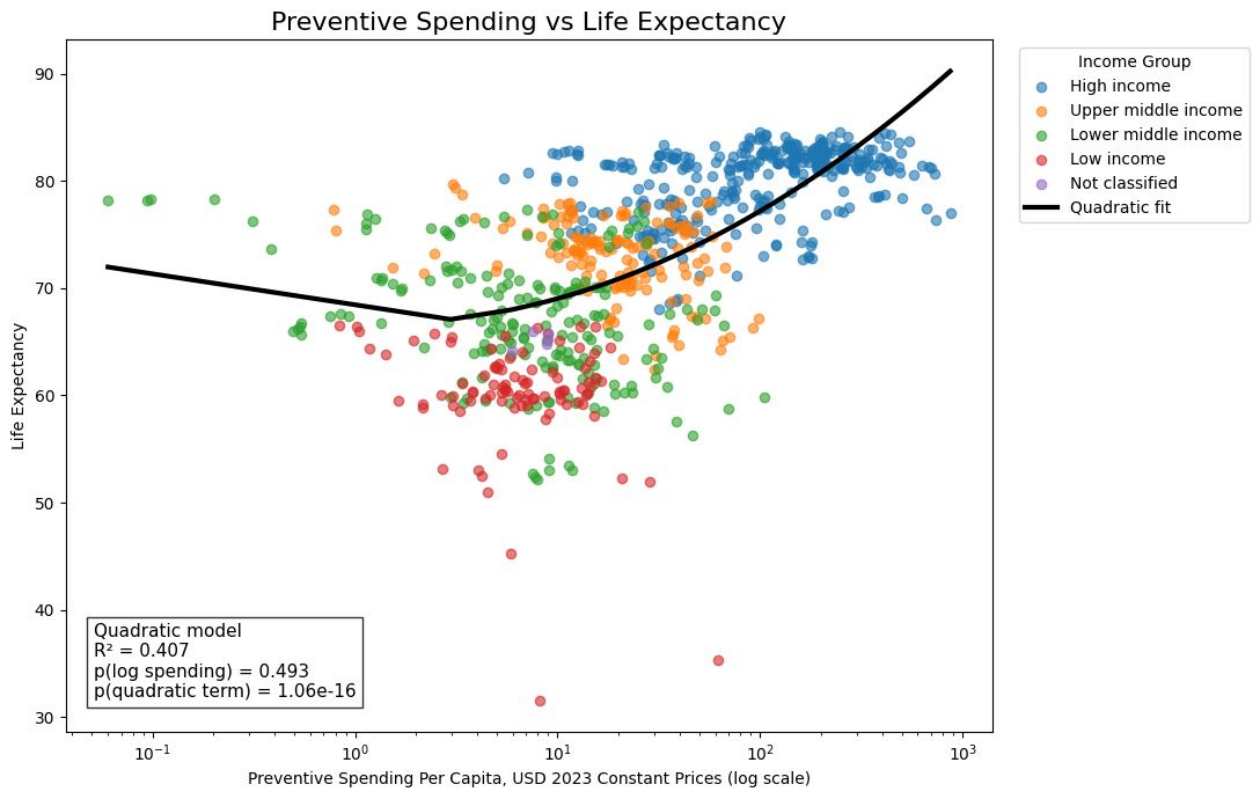
Same Share, Different Resources



- Countries with **similar prevention shares** can still spend dramatically **different amounts per person**.
- Some low-income countries devote **large shares** toward prevention but operate with very **limited resources**.
- High-income countries may allocate **smaller shares** while still investing vastly **more actual funding**.

Relative **percentages** can therefore **hide major inequalities** in healthcare capacity.

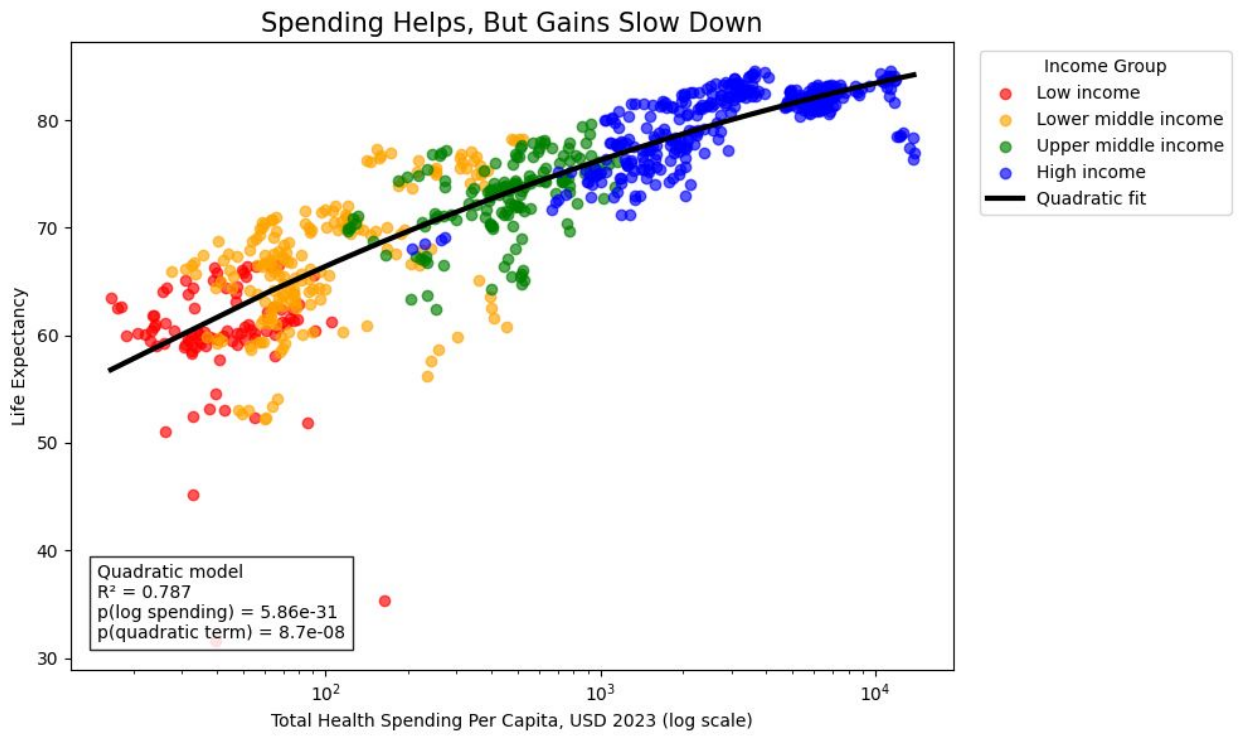
More Prevention Spending Per Capita, Longer Lives



- Countries spending more actual money **per person** on prevention tend to achieve **longer life expectancy**
- The curve rises **faster** at **higher spending levels** in this model.
- High-income countries **cluster** at high spending and high life expectancy.

This suggests **healthcare capacity** plays a major role in shaping outcomes.

Spending Helps, But Gains Slow Down



- **Overall healthcare spending** per capita is **strongly associated** with higher life expectancy across countries.
- The relationship is nonlinear, with additional healthcare investment producing **smaller improvements** in life expectancy at **high levels**.

This suggests that healthcare outcomes depend not only on spending amount, but also on how efficiently healthcare systems **convert spending into outcomes**.